

Nivolumab + ipilimumab + pemetrexed + cisplatin

Lung - NCCP 00714 - version 4

Your medicines

Nivolumab - Ipilimumab - Pemetrexed - Cisplatin

Your treatment schedule

Nivolumab + ipilimumab + pemetrexed + cisplatin.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Nivolumab / ipilimumab - immune effects

1 Hormones / brain

Nivolumab / ipilimumab

Fatigue, dizziness, headache, thirst or passing more urine can indicate immune endocrine toxicity.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, palpitations, fainting or marked muscle weakness needs urgent assessment.

5 Kidneys / hydration

Pemetrexed / cisplatin

Kidney function is monitored closely and cisplatin requires hydration/electrolyte support.

7 Blood / infection

Pemetrexed / cisplatin

Fever, shivers, bruising or sudden illness can reflect low blood counts.

Pemetrexed / cisplatin - chemotherapy

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment for pneumonitis and other causes.

4 Liver / bowel

Nivolumab / ipilimumab

Jaundice, dark urine, persistent diarrhoea or abdominal pain can indicate immune inflammation.

6 Hearing / nerves

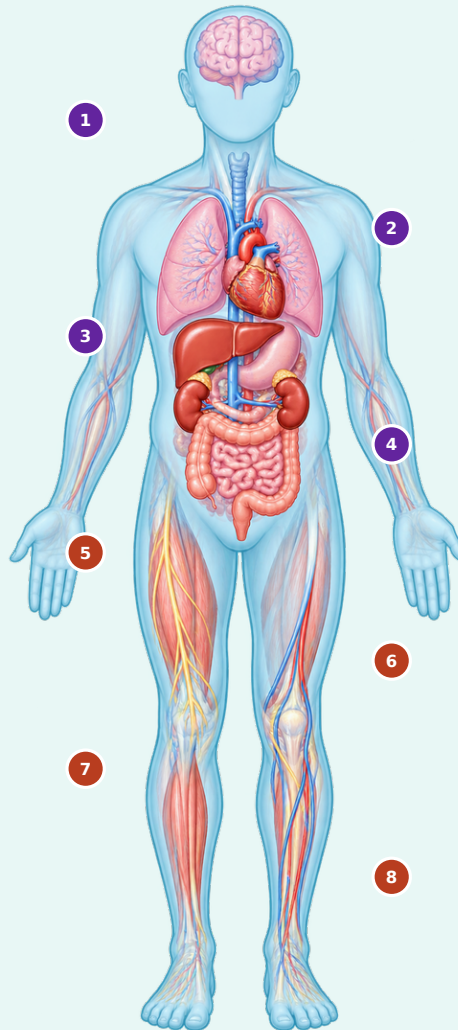
Cisplatin

Report tinnitus, hearing change, numbness or tingling before it worsens.

8 Nausea / mouth

Cisplatin / pemetrexed

Use the prescribed anti-sickness medicines; persistent vomiting or severe mouth soreness needs advice.



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

- Cycle 1 - day 1: Nivolumab + ipilimumab + pemetrexed + cisplatin.
- Cycle 1 - day 22: Nivolumab + pemetrexed + cisplatin.
- After cycle 1: Continue nivolumab on days 1 and 22 and ipilimumab on day 1 of each 42-day cycle.
- Duration: Immunotherapy continues until progression/unacceptable toxicity or up to 24 months in patients without progression, according to the NCCP source.

Monitoring and support

- Folic acid and vitamin B12 support are required with pemetrexed.
- Cisplatin requires hydration/electrolyte management and high-emetogenic-risk antiemetic support.
- Blood count, kidney/liver profile and endocrine monitoring support treatment safety.
- Immune adverse events can begin during treatment or after it stops; report new symptoms early.

WHEN TO CALL

Contact oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, chest pain, marked weakness, inability to maintain fluids, reduced urine or new neurological symptoms. For severe breathing difficulty, collapse, seizure or severe chest pain call 112/999.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00714/>
Official NCCP 00714 v4
SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

